

THE

Geriatrician's Guide

A PREPARATION FRAMEWORK

Starting the Conversation

The most important conversation in this whole process isn't with your siblings, or the financial advisor, or the admissions director.

It's with your loved one — the person at the center of it all.

This guide won't script that conversation. What it will do is help you walk in prepared.



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No placement commissions.
No facility money. Ever.

WHAT THIS IS

This is a short framework for one specific conversation: telling your loved one it may be time to consider senior living, and inviting them to start thinking about it with you.

It's built from what I've seen work, and backfire, across years of watching these conversations happen — in exam rooms and in living rooms alike.

HOW TO USE IT

Read it once straight through before your first conversation. The six steps build on each other in the order they tend to come up.

Then keep it close. Most people need more than one conversation — before each one, reread whichever step matches what's ahead of you next.



ABOUT THE AUTHOR

The Geriatrician's Guide is founded and edited by **Michael D. Genualdi, MD**, a board-certified geriatrician and internist. After completing his residency and fellowship training at Stanford, he spent years caring for older adults across the full range of care settings — including serving as medical director of a primary care practice embedded in dozens of senior living communities. He built this resource to share what that experience has taught him, so people can care for a loved one with confidence instead of guesswork. The Geriatrician's Guide does not accept compensation of any kind from senior living operators, placement services, or referral networks.

01 Choose the moment — when you can.

Not when either of you is rushed, tired, or raw — a calm, intentional setting with no clock running gives an honest conversation the best chance.

Many people don't get the choice. The decision gets forced by a fall, a hospitalization, or a new diagnosis, and the conversation happens in a hospital hallway on someone else's schedule. That's the case for starting now, while you can still pick the moment.



THE UNCOMFORTABLE STEP

The discomfort of an early conversation is nothing compared to the one a crisis chooses for you.

02 Go in to listen, not to convince.

The goal of the first conversation isn't a final decision, scheduling a tour, or even agreement. It's to open a door and leave it open. Walk in needing a specific outcome and you'll push, they'll pull back, and you'll both leave more entrenched than you started.

Approach with genuine curiosity about what your loved one is afraid of. Losing their home. Losing their independence. Losing their sense of who they are. Becoming a burden to you. These aren't irrational objections to argue past — they're legitimate fears, and the people who do this well take them seriously and work to address them directly, instead of treating them as resistance to get through.

03 Name the feeling before the facts.

Lead with what you're experiencing, not with evidence of what they're doing wrong. The same words land differently when the person hearing them doesn't feel accused.

When someone feels accused or criticized, the brain's threat response activates — the same state that makes people defensive, and shuts the conversation down completely.

✓ SAY THIS — KEEPS THE DOOR OPEN

"I'm worried about your safety."

"Your dignity matters to me."

"This is becoming more than I can manage alone."

✗ NOT THIS — SLAMS IT SHUT

"You left the stove on again."

"You're not keeping yourself clean anymore."

"You're a burden to me."

*The right-hand column may be true. **That's not the point.** Lead with the left.*

04 Bring the assessment — don't lead with it.

If you've worked through the *Is It Time?* self-assessment, treat it as a tool, not a weapon. Don't set it down as a verdict you're delivering. Offer it as something you did yourself — "I went through this, and here's what came up for me" — and invite them to do it too, separately, then compare.

If the two of you score the same situation differently, **that gap is the useful part**: it shows you where you actually disagree, which is where the real conversation starts.

BEFORE YOUR NEXT CONVERSATION

Write down your actual first sentence. Not a general intention, not what you want to accomplish, but the specific words you'll say first — and make sure they lead with your emotions, not their failures.

05 Expect more than one conversation.

The first conversation rarely resolves anything, and it doesn't need to. Planting the idea and letting it sit usually works better than pushing for an answer in one sitting. Each conversation builds on the last. You're trying to keep the discussion open — long enough to make a good decision together, before a crisis makes it for you.

06 When you're met with resistance.

Sometimes the answer is no. It happens, and it's hard. Three things help.

Separate safety from preference.

A person has every right to their preferences — but not to a choice that puts them or others in danger. Naming that line clearly, without threatening, sometimes shifts the conversation.

Bring in a trusted third voice.

A physician, a faith leader, an old friend. The same words may land differently coming from a third party.

Know the difference between refusing and not understanding.

Someone who grasps the situation and says no is exercising their right. Someone who genuinely can't take in what's happening is a different matter — that's a medical question about capacity, and it changes your approach. If you're not sure which one you're seeing, raise it with their doctor.

IN CLOSING

You don't have to get it perfect. You just have to start — on your terms, before a crisis sets them for you. Do that, and the hardest part is behind you.

I hope this gives you what you need to walk in prepared.

Once you're ready to take the next step — the questions become when, where, and how much. I built a resource to help you answer them.

— Dr. Michael Genualdi

YOUR NEXT STEP

The Tour Day Kit: A Geriatrician's Guide to Evaluating Senior Living Communities

The 57 questions I would ask on a community tour, a pocket card to bring inside, a notes sheet for what you hear, and a scorecard to compare communities side by side. Equip yourself for one of the most consequential decisions you and your loved one will make.

[Get the Tour Day Kit →](#)

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